St. Jude Academic Health Center - Division of Hospital Medicine

CLINICAL DOCUMENTATION — NARRATIVE EMERGENCY OBSERVATION NOTE V1

Patient: David Miller | **MRN:** PAT-60007 | **DOB:** 1962-03-08 | **Gender:** Male | **Doc ID:** NARRATIVE_0007
Encounter Date: 2025-03-21 | **Discharge/Note Date:** 2025-03-26 | **Provider:** Dr. Gregory House, MD

EMERGENCY DEPARTMENT PRESENTATION

Patient David Miller was triaged in the emergency department following acute onset of lightheadedness and palpitations earlier this morning. Initial hemodynamic parameters were reviewed and patient was placed on telemetry.

HISTORY OF PRESENT ILLNESS

The patient describes experiencing recurrent lightheadedness over the preceding 48 hours without syncope. Initial emergency workup revealed that initial platelet count was measured at 245.5 10*3/uL. Point-of-care laboratory screening additionally noted potassium resulted at 6.03 mmol/L (HIGH). Given these findings, the patient was transitioned to the Clinical Decision Unit for 24-hour observation.

OBSERVATION UNIT CLINICAL COURSE

Throughout the observation unit stay, continuous telemetry showed normal sinus rhythm without pause or tachyarrhythmia. The patient was administered Ciprofloxacin at a dose of 250 mg PO every 12 hours for 7 days, which promptly alleviated acute symptoms. Follow-up protocol monitoring of hemoglobin demonstrated 13.5 g/dL. Orthostatic blood pressures were negative upon ambulation.

DISCHARGE REGIMEN

Patient was deemed safe for outpatient discharge. The ongoing regimen includes Metoprolol 25 mg oral twice daily as prescribed. Prescription counseling was performed by nursing prior to release.

PLAN & PRECAUTIONS

Resume normal daily activities as tolerated. Avoid strenuous exercise until cardiology follow-up in 1 to 2 weeks. Immediate emergency precautions detailed.